

Hiccups

By [Jonathan Gotfried](#), MD, Main Line Health, Bryn Mawr, PA
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Hiccups are repeated involuntary spasms of the diaphragm, followed by quick, noisy closings of the glottis. The diaphragm is the muscle that separates the chest from the abdomen and that is responsible for each breath. The glottis is the opening between the vocal cords, which closes to stop the flow of air to the lungs.

Brief episodes of hiccups (lasting a few minutes) are very common. Occasionally, hiccups persist for some time, even in healthy people. Sometimes hiccups can last more than 2 days or even more than 1 month. These longer episodes are called persistent hiccups. Persistent hiccups are uncommon but can be quite distressing.

Hiccups are more common among men.

Causes of Hiccups

Doctors are not clear why hiccups happen but they think it may involve irritation of the nerves or the parts of the brain that control muscles of respiration (including the diaphragm).

Brief episodes of hiccups often have no obvious cause but sometimes are triggered by:

- A bloated stomach
- Alcohol consumption
- Swallowing hot or irritating substances

In such cases, hiccups usually start in a social situation, perhaps triggered by some combination of laughing, talking, eating, and drinking (particularly alcohol). Sometimes hot or irritating food or liquids are the cause. Hiccups are more likely to occur when carbon dioxide levels in the blood decrease. Such a decrease can occur when people hyperventilate.

Persistent episodes of hiccups sometimes have more serious causes (see table [Some Causes and Features of Persistent Hiccups](#)). For example, the diaphragm may become irritated because of pneumonia, chest or stomach surgery, or waste products that accumulate in the blood when the kidneys malfunction (uremia). Rarely, hiccups develop when a [brain tumor](#) or [stroke](#) interferes with the breathing center in the brain.

When the cause is serious, hiccups tend to persist until the cause is corrected. Hiccups due to a brain tumor or stroke may be very hard to stop and may become exhausting.

Evaluation of Hiccups

Brief episodes of hiccups do not require evaluation by a doctor. For persistent hiccups, the following information can help people decide whether a doctor's evaluation is needed and help them know what to expect during the evaluation.

Warning signs

In people with hiccups, certain symptoms and characteristics are cause for concern. They include:

- Neurologic symptoms (such as headache, weakness, numbness, and loss of balance)

When to see a doctor

People who have hiccups and warning signs should see a doctor right away.

People without warning signs should see a doctor if hiccups last more than 2 or 3 days.

What the doctor does

Doctors first ask questions about the person's symptoms and medical history. Doctors then do a physical examination. What doctors find during the history and physical examination often suggests a cause of the hiccups and the tests that may need to be done (see table [Some Causes and Features of Persistent Hiccups](#)).

The history is focused on how long the hiccups have lasted, what remedies the person has tried, and whether the person has recently been ill or had surgery. Doctors also ask people whether they have any

- Symptoms of gastroesophageal reflux

- Swallowing difficulties
- Cough, fever, or chest pain
- Neurologic symptoms (such as headaches and/or difficulty walking, talking, speaking, or seeing)

Doctors also ask people about their use of alcohol.

The physical examination is focused on a full neurologic examination. A general examination usually does not reveal much, but doctors look for signs of chronic disease such as severe wasting away of muscle and fat tissue (cachexia).



Some Causes and Features of Persistent Hiccups

Cause	Common Features*	Tests
Esophagus		
<u>Gastroesophageal reflux disease</u>	Heartburn (burning pain that begins in the upper abdomen and travels up to the throat, sometimes with an acid taste in the mouth)	A doctor's examination
	Chest pain	Sometimes trying treatment with medications to suppress acid production
	Sometimes a cough, hoarseness, or both	Sometimes endoscopy of the upper digestive tract (examination of the esophagus and stomach using a flexible viewing tube)
	Symptoms sometimes triggered by lying down	
	Relief with antacids	
Abdomen		
Abdominal surgery (recent)	Obvious history of recent surgery	A doctor's examination
<u>Gallbladder disease</u>	Pain in the upper right part of the abdomen, under the rib cage	<u>Ultrasound</u>
	Sometimes nausea and vomiting	
<u>Hepatitis</u>	A general feeling of illness (malaise)	Blood tests
	Poor appetite	
	Nausea and sometimes vomiting	
	Sometimes darkening of the urine, then yellowing of the skin	



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